

Response to HHS RFI: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Strategic Summary

This submission responds to the HHS Request for Information by outlining a decision-architecture approach designed to operate under conditions of high uncertainty, fragmented data, and accelerated policy timelines. The approach focuses on structuring multi-domain inputs into transparent decision pathways that preserve traceability, governance, and human oversight while increasing decision velocity and outcome confidence. It is modular, non-prescriptive, and intended to augment—not replace—existing HHS systems, processes, and authorities.

Submission Context

This response reflects cross-sector experience designing, governing, and evaluating AI-enabled decision systems in complex, regulated healthcare environments. The comments focus on adoption at scale—specifically the governance, evaluation, and incentive structures required to move clinical AI from pilots into routine care while preserving trust, safety, and fiscal integrity.

Framing Statement (Decision Spine)

Clinical AI is not constrained by technical capability; it is constrained by decision governance. Adoption at scale requires continuous accountability, auditable decision-making, and alignment across regulation, reimbursement, evaluation, and clinical workflows.

Primary Levers (1–3)

- **Governance:** Clarify accountability for non-device AI through continuous oversight expectations.
- **Evaluation:** Normalize post-deployment monitoring as operational infrastructure, not a one-time event.
- **Incentives:** Align reimbursement with monitored, real-world performance to sustain adoption and protect program integrity.

Observed Barriers to Adoption

The most significant barriers are governance ambiguity, workflow disruption, and incentive misalignment rather than model performance. Providers lack clear expectations for liability, auditability, post-deployment monitoring, and human oversight for non-device AI. This uncertainty increases administrative burden, slows internal decision-making, and keeps AI trapped in pilots.

In practice, fragmented governance often results in duplicative reviews across compliance, legal, IT, and clinical leadership—adding weeks or months to adoption timelines. AI is also frequently introduced as a parallel system rather than embedded within existing workflows, increasing documentation time and clinician resistance.

Regulatory, Reimbursement, and Programmatic Actions

HHS can accelerate adoption by clarifying acceptable governance patterns for non-device AI rather than regulating specific technologies. Guidance should define baseline expectations for human oversight, documentation and traceability of AI-influenced decisions, post-deployment monitoring, and conditions for rollback or suspension.

On reimbursement, HHS could pilot payment pathways explicitly tied to monitored outcomes and operational performance, reducing the risk of waste, fraud, and abuse while rewarding demonstrated value. Programmatically, HHS should emphasize implementation science and real-world evaluation infrastructure alongside innovation.

Governance, Liability, Privacy, and Security for Non-Device AI

Non-device AI often influences care decisions without fitting existing device frameworks, creating gaps in accountability and oversight. Challenges include unclear responsibility for outcomes, insufficient audit trails, limited standardized monitoring, and patient uncertainty regarding consent and recourse.

A decision-centric governance approach—focused on traceability, continuous monitoring, defined human authority boundaries, and privacy-by-design—can close these gaps without constraining innovation.

Evaluation and Monitoring as Infrastructure

Pre-deployment testing is necessary but insufficient. Safe scale requires continuous post-deployment monitoring, including performance drift detection, outcome tracking, bias and equity monitoring, and workflow impact measurement.

For example, a clinical decision-support system that performs well at deployment may drift as patient populations, coding practices, or care protocols change. Continuous monitoring enables early detection, targeted retraining, or suspension before harm or reimbursement risk occurs.

HHS can catalyze adoption by standardizing evaluation frameworks and shared metrics that make ongoing monitoring a norm rather than an exception.

Certification, Accreditation, and Credentialing

Process-based certification is more scalable than model-specific approval. HHS can support accreditation of governance and monitoring practices, certification of evaluation and audit processes, and credentialing focused on operational readiness. This approach preserves innovation while ensuring consistent safeguards across care settings.

Interoperability and Benchmarking

Interoperability efforts should prioritize benchmarking and evaluation, not just data exchange. Standardized outcome metrics, comparable evaluation datasets, and common reporting structures for post-deployment monitoring will expand markets, reduce friction, and accelerate learning across systems.

Patient and Provider Perspectives

Patients and caregivers seek transparency into how AI influences care, meaningful consent, assurance of privacy, and clear recourse when errors occur. Providers want tools that reduce burden rather than add parallel workflows—measurably decreasing documentation time, review cycles, and administrative overhead.

Trust increases when AI supports—rather than replaces—human decision-making and accountability remains with care teams.

Research Priorities

HHS research investments should prioritize implementation science, real-world evaluation, and governance models over incremental model novelty. High-impact areas include post-deployment monitoring methods, human–AI interaction in workflows, governance for non-device AI, and mechanisms aligning reimbursement with monitored outcomes.

Closing Statement

HHS’s greatest leverage is not selecting technologies, but shaping the governance and incentive environment that allows trustworthy AI to move from pilots to policy to execution — safely, measurably, and at scale.

Key Takeaways

- **Problem:** Clinical AI adoption stalls due to governance ambiguity, administrative burden, and misaligned incentives.
 - **Core Insight:** Decision-centric governance with continuous evaluation and outcome-aligned reimbursement.
 - **Result:** Safe, scalable, and fiscally responsible AI adoption in clinical care.
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The submitter is available to provide clarification or additional context if helpful to HHS’s ongoing analysis.

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